A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Valerie Tay
Suite 413

Dear Dr Tay,

Re: Ms Gordana Vukobrat
DOB: 1980-03-22
Address: 80 Perkins Road, LARA VIC 3212

Summary

- **Persistent nausea for investigation via gastroscopy.**
- **CT abdomen demonstrating a benign hepatic cyst and uterine lesion likely representing known adenomyomatosis.**
- **Coeliac disease excluded despite borderline antibody elevation.**
- **No action required by you.**

I had the pleasure of reviewing Gordana in the clinic to discuss her ongoing gastrointestinal symptoms and recent investigations. Gordana continues to experience nausea approximately 80% of the time. While she lost approximately five kilograms earlier this year following an episode of gastroenteritis, she has since regained weight and her clinical status remains stable with no reports of vomiting.

Her recent haematology and biochemistry results were largely unremarkable. While her Vitamin B12 is at the lower end of the normal range and the RDW is marginally elevated, these findings are not clinically significant at this stage. We discussed her coeliac serology, which showed a borderline Tissue Transglutaminase (tTG) antibody level of 21. Given her negative genetic testing and the minimal nature of this elevation, I have advised her that a diagnosis of coeliac disease is highly improbable. She notes some symptomatic improvement when reducing gluten intake; I have encouraged this as a subjective symptom management strategy rather than a clinical necessity for mucosal healing.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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We also reviewed the images from her recent CT scan. The scan identified a small hepatic cyst and a uterine lesion. Gordana has a known history of fibroids and adenomyomatosis, and as she underwent a pelvic ultrasound within the last six months, I do not believe further pelvic imaging is required at this time. The hepatic cyst is a benign incidental finding.

Impression and Plan Gordana's persistent nausea warrants further evaluation via a gastroscopy to exclude upper gastrointestinal pathology. We discussed the procedure in detail, specifically addressing her reservations regarding anaesthesia. I have recommended the use of propofol sedation to ensure a thorough examination and to mitigate discomfort. Gordana has agreed to this plan, and I will arrange for the procedure to be performed at St Vincent's Hospital. I will review her again once the gastroscopy and associated biopsies are completed.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Elissa Armitage

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